Cranial Nerve Examination — OSCE Checklist

Deakin MD2 · Neuro · Time: 8 min (+2 min reading)

Tick each step as you perform or verbalise it. **Shaded rows (*)** *are usually verbalised in the OSCE ("I would also…"). Compare* **left vs right** *throughout and narrate aloud.*

1. Introduction (WIPER)

☐	**W**ash hands / hand hygiene
☐	**I**ntroduce self (name, role)
☐	**P**atient ID: name and DOB
☐	**E**xplain the exam and gain consent
☐	**P**osition: seated, eye level, ~1 m apart
☐	Ask about **pain** before starting
☐	Equipment ready: pen torch, Snellen, red pin, ophthalmoscope, cotton wool, neurotip, 512 Hz fork, tongue depressor

2. General inspection (end of bed, 10–15 s)

	Look for	Significance
☐	Well or unwell	Distress, drowsiness, airway
☐	Bedside clues	Walking aids, NG tube, thickened fluids
☐	Head shape	Craniotomy scar, shunt, Paget's
☐	Ptosis / proptosis, pupils, gaze	CN III, Horner's, thyroid eye disease
☐	Facial asymmetry	Droop, flat nasolabial fold (CN VII / stroke)
☐	Scars, cachexia, rashes, skin lesions	Parotid/neck scars, ear vesicles (Ramsay Hunt)
☐	Hearing aids, glasses	Baseline sensory deficit
☐	Gait and posture	Ataxia, hemiplegic gait

3. Cranial nerves I–XII

	Step	What to do / say	Abnormal
CN I — Olfactory			
☐	Smell*	"Any change in smell or taste?" Formal test one nostril at a time	Anosmia: URTI, trauma, Parkinson's
CN II — Optic			
☐	Visual acuity	Snellen at 6 m, glasses on, each eye covered; record e.g. 6/6	Pinhole if reduced
☐	Visual fields	1 m apart, cover opposite eyes, red pin from 4 quadrants	Bitemporal / homonymous hemianopia
☐	Fundoscopy*	Ophthalmoscope: disc, vessels, macula	Papilloedema, optic atrophy
CN II + III — Pupils			
☐	Inspect pupils	Size, shape, symmetry	Anisocoria
☐	Direct + consensual	Light from the side into each eye	Absent = afferent/efferent defect
☐	Swinging light	Swing torch between eyes	RAPD (Marcus Gunn)
☐	Accommodation	"Look far away, now at my finger"	Constrict + converge
CN III, IV, VI — Eye movements			
☐	H test	Head still, follow finger slowly in an H; ask about diplopia	III: down + out, ptosis · IV: vertical diplopia · VI: no abduction
☐	Nystagmus	Hold at extremes of gaze	Cerebellar / vestibular
CN V — Trigeminal			
☐	Sensation	Cotton wool (± neurotip) V1 forehead, V2 cheek, V3 jaw; eyes closed	Loss in a division
☐	Motor	Palpate masseter + temporalis on clenching; open jaw against resistance	Jaw deviates to weak side
☐	Corneal reflex*	Cotton wisp to cornea from the side	No blink (V afferent, VII efferent)
☐	Jaw jerk	Finger on chin, mouth slightly open, tap	Brisk = UMN
CN VII — Facial			
☐	Raise eyebrows	"Look up at the ceiling"	Forehead spared = UMN
☐	Eyes shut tight	"Don't let me open them"	Weak closure
☐	Show teeth	"Show me your teeth"	Asymmetric smile
☐	Puff cheeks	"Blow out your cheeks" — press gently	Air escapes on weak side
CN VIII — Vestibulocochlear			
☐	Whisper test	Rub opposite tragus; whisper a number (e.g. 68, 100) at arm's length	Can't repeat = hearing loss
☐	Rinne	512 Hz fork on mastoid, then beside ear: "Which is louder?"	BC > AC = conductive

	Step	What to do / say	Abnormal
☐	Weber	Fork on vertex: "Where do you hear it?"	Lateralises (see box)
CN IX, X — Glossopharyngeal, Vagus			
☐	Palate	"Say Ahh" — palate rises symmetrically; ask about swallow, voice, cough	Uvula deviates AWAY from lesion
☐	Gag reflex*	Verbalise	Absent: IX sensory / X motor
CN XI — Accessory			
☐	Shrug shoulders	Against resistance (trapezius)	Weak on lesion side
☐	Turn head	Against resistance; feel opposite SCM	Weak turn AWAY from lesion side
CN XII — Hypoglossal			
☐	Tongue at rest	Inspect in mouth for wasting, fasciculations	LMN lesion
☐	Protrude + side to side	"Stick out your tongue, move it side to side"	Deviates TOWARDS lesion

4. Closing and presenting

☐	Thank patient, reposition, wash hands
☐	Completion statement: "To complete, I would perform fundoscopy, corneal and gag reflexes, formal smell/taste, a full peripheral neuro exam and audiometry if indicated."
☐	Present: "I examined Mr X, a [age]-year-old… Cranial nerves I–XII were grossly intact. This is a normal cranial nerve examination." If abnormal, lead with the key finding + likely site of lesion.

5. Quick reference

Hearing	Rinne	Weber
Normal	AC > BC both ears	Central
Conductive	BC > AC affected ear	TO affected ear
Sensorineural	AC > BC (both reduced)	AWAY from affected ear

Facial palsy	UMN (stroke)	LMN (Bell's)
Forehead	Spared	Involved
Eye closure	Preserved	Weak, Bell's phenomenon
Lesion side	Contralateral	Ipsilateral

- **Uvula** deviates AWAY from the lesion; **tongue** points TOWARDS it.
- **Ptosis + small pupil** = Horner's · **ptosis + dilated pupil + down-and-out** = CN III (pupil involved: think PComm aneurysm).
- **RAPD:** affected pupil paradoxically dilates when light swings to it = optic nerve lesion.
- **Bulbar (LMN IX–XII):** wasted, fasciculating tongue (MND) · **Pseudobulbar (UMN):** spastic tongue, brisk jaw jerk, emotional lability.
- **CP angle lesion** (acoustic neuroma): V, VII, VIII together; lost corneal reflex is early.
- **Pitfalls:** not asking about pain/diplopia, moving finger too fast, patient moving head, not comparing sides, examining silently.

Upper Limb Neurological Exam — OSCE Checklist

Deakin MD2 · BB6/BB7 · Time: 8 min (+2 min reading)

Tick each step as you perform or verbalise it. **Shaded rows (*)** *are optional or done if indicated. Order:* **Inspection → Tone → Power → Reflexes → Coordination → Sensation**. *Always compare* **left vs right** *and narrate aloud.*

1. Introduction (WIPER)

☐	**W**ash hands / hand hygiene
☐	**I**ntroduce self (name, role)
☐	**P**atient ID: name and DOB
☐	**E**xplain the exam and gain consent
☐	**E**xpose: top half exposed (singlet / sports bra); maintain dignity, good lighting
☐	Ask about **pain** before touching
☐	Equipment ready: tendon hammer, cotton wool, neurotip (sharps safety, gentle on thin skin), 128 Hz tuning fork

2. Examination

	Step	What to do / say	Root / abnormal
Inspection			
☐	General	Posture, walking aids, scars, skin changes	Hemiplegic posture, deformity
☐	Muscle bulk	Both sides: shoulder girdle, arms, thenar/hypothenar eminences, dorsal interossei	Wasting = LMN; symmetry
☐	Fasciculations	Watch muscles at rest for 10–20 s	LMN (e.g. MND)
☐	Abnormal movements	Tremor (rest/action), myoclonus, chorea, dystonia	Resting tremor = Parkinson's
☐	Pronator drift	"Arms out, palms up, eyes closed" for 20–30 s	Pronates + drifts down = UMN
Tone			
☐	Wrist + elbow	Ask about pain; patient relaxed; passively flex/extend and pronate/supinate at variable speed	Normal / increased / decreased
☐	Rigidity	Feel for cogwheeling (+ tremor) or lead-pipe rigidity	Extrapyramidal (Parkinson's)
☐	Spasticity	Velocity-dependent catch (clasp-knife)	UMN
Power (MRC 0–5) — isolate the joint, match the patient's strength			
☐	Shoulder abduction	"Lift your arms up like wings, don't let me push down"	C5, C6 — deltoid, supraspinatus
☐	Shoulder adduction	"Push your arms down to your sides"	C6, C7, C8 — pec major, lat dorsi
☐	Elbow flexion	"Pull your arm in towards you"	C5, C6 — biceps, brachialis
☐	Elbow extension	"Push me away"	C7, C8 — triceps
☐	Wrist flexion	"Push your wrists down, don't let me push them up"	C6, C7 — FCR, FCU
☐	Wrist extension	"Push your wrists up, don't let me push them down"	C7, C8 — ECR, ECU
☐	Finger extension	"Straighten your fingers, don't let me bend them"	C7, C8 — extensor digitorum
☐	Finger flexion	"Squeeze my two fingers"	C7, C8 — FDP, FDS
☐	Finger abduction	"Spread your fingers, don't let me push them together"	C8, T1 — dorsal interossei
☐	Finger adduction	Resist examiner separating fingers (paper grip)	C8, T1 — palmar interossei
☐	Thumb opposition*	"Touch your thumb to your little finger"	Median nerve (T1)
Reflexes — relaxed, positioned; reinforce (clench teeth) if absent			
☐	Biceps	Arm in lap, forearm pronated; finger on tendon, strike finger	C5, C6
☐	Brachioradialis	Fingers on lateral radius just above wrist; strike	C5, C6
☐	Triceps	Arm loosely supported across chest; strike tendon above olecranon	C7, C8
☐	Finger jerk*	Hand supinated, fingers slightly flexed; strike your own fingers on theirs	C8
☐	Hoffmann's*	Flick distal phalanx of middle finger; watch thumb	Thumb flexes = UMN
Coordination			
☐	Finger–nose	"Touch my finger, then your nose", move target; repeat each side	Past-pointing, intention tremor
☐	Rapid alternating	Pat palm with front then back of the other hand, fast	Dysdiadochokinesia
☐	Rebound	Outstretched arms, eyes closed; tap down — arm returns	Overshoot = cerebellar
Sensation — demonstrate on sternum first, eyes closed, compare sides			
☐	Light touch	Cotton wool: dab each dermatome C5–T1	Dermatomal / glove pattern
☐	Pain	Neurotip: sharp or blunt? Same dermatomes	Spinothalamic loss
☐	Temperature*	Cold tuning fork if indicated	Spinothalamic
☐	Vibration	128 Hz fork on DIP; if absent move to MCP → wrist → elbow	Dorsal column loss
☐	Proprioception	Hold DIP by its sides; "Up or down?"; if absent move proximally	Dorsal column / sensory ataxia



Proprioception: hold the sides of the distal phalanx

3. Closing and presenting

☐	Thank patient, help them dress, wash hands
☐	Completion statement: "To complete, I would examine the lower limbs and cranial nerves, and check the neck/spine."
☐	Present one finding per domain: "On inspection normal bulk with no fasciculations; tone normal; power 5/5 throughout; reflexes normal and symmetrical; coordination normal; sensation intact. Overall a normal upper limb neurological exam."

4. Quick reference

MRC	Power grade
0	No contraction
1	Flicker, no movement
2	Moves with gravity eliminated
3	Moves against gravity, not resistance
4	Against resistance, weaker than normal
5	Normal power

Dermatome	Test site
C5	Shoulder tip / lateral upper arm
C6	Lateral forearm, thumb
C7	Middle finger
C8	Little finger
T1	Medial upper arm / elbow
Reflexes	0 absent · 1+ reduced · 2+ normal · 3+ brisk · 4+ clonus

Sign	UMN	LMN
Bulk	Normal / disuse	Wasting
Fasciculations	No	Yes
Tone	Increased (spastic)	Decreased
Power	Pyramidal weakness (extensors)	Weak in root/nerve distribution
Reflexes	Brisk, Hoffmann's +	Reduced / absent

Nerve	Classic finding
Radial	Wrist drop; sensory loss first dorsal web space
Ulnar	Claw hand (4th/5th), interossei wasting, Froment's +
Median	Thenar wasting, weak opposition; Tinel's / Phalen's (carpal tunnel)

- **Pyramidal pattern (UMN):** extensors weaker than flexors in the arm — shoulder abduction, elbow and wrist extension go first.
- **Cerebellar signs (DANISH):** Dysdiadochokinesia, Ataxia, Nystagmus, Intention tremor, Slurred speech, Hypotonia — ipsilateral to lesion.
- **Glove distribution** sensory loss = peripheral neuropathy (diabetes, B12, alcohol); **dermatomal** = root; **nerve territory** = mononeuropathy.
- **Pitfalls:** overpowering the patient, not isolating the joint, triceps tested with arm unsupported, skipping reinforcement, testing sensation with eyes open.

Lower Limb Neurological Exam — OSCE Checklist

Deakin MD2 · BB6/BB7 · Time: 8 min (+2 min reading)

Tick each step as you perform or verbalise it. **Shaded rows (*)** *are optional or done if indicated. Order:* **Inspection + gait → Tone → Power → Reflexes → Coordination → Sensation**. *Always compare* **left vs right** *and narrate aloud.*

1. Introduction (WIPER)

☐	**W**ash hands / hand hygiene
☐	**I**ntroduce self (name, role)
☐	**P**atient ID: name and DOB
☐	**E**xplain the exam and gain consent
☐	**E**xpose: legs exposed (underwear / shorts), cover groin; maintain dignity, good lighting
☐	Ask about **pain** before touching
☐	Equipment ready: tendon hammer, cotton wool, neurotip (sharps safety, gentle on thin skin), 128 Hz tuning fork

2. Examination

	Step	What to do / say	Root / abnormal
Inspection + gait (patient standing first)			
☐	General	Walking aids, orthoses, footwear wear, scars (back, legs), skin	Deformity, pes cavus
☐	Muscle bulk	Both sides: quadriceps, calves, tibialis anterior, small foot muscles	Wasting = LMN; symmetry
☐	Fasciculations	Watch thighs and calves at rest	LMN (e.g. MND)
☐	Gait	Walk to wall and back: arm swing, stride, base, turning	Hemiplegic, shuffling, ataxic, high-stepping
☐	Heel walk	"Walk on your heels"	Can't = L4/L5, foot drop
☐	Toe walk	"Walk on your tiptoes"	Can't = S1
☐	Tandem walk	"Heel to toe in a straight line"	Ataxia (cerebellar)
☐	Romberg's	Feet together, arms by side, eyes open then closed; stand close to catch	Sway with eyes closed only = sensory ataxia
Tone (patient lying on bed)			
☐	Leg roll	Roll the leg side to side; foot should flop	Stiff = increased tone
☐	Knee lift	Quickly lift behind knee	Heel lifts off bed = increased tone
☐	Knee + ankle	Passively flex/extend at variable speed	Spasticity / rigidity / hypotonia
☐	Ankle clonus	Knee slightly bent; sharply dorsiflex and hold	≥ 3–5 beats = UMN
Power (MRC 0–5) — isolate the joint, match the patient's strength			
☐	Hip flexion	"Keep your leg straight, lift it up, don't let me push it down"	L2, L3 — iliopsoas
☐	Hip extension	"Push my hand down into the bed" (hand behind calf)	L5, S1, S2 — gluteus maximus
☐	Hip abduction	"Push your legs out against my hands"	L4, L5, S1 — gluteus medius/minimus
☐	Hip adduction	"Squeeze your legs together"	L2, L3, L4 — adductors
☐	Knee flexion	Knee bent: "Pull your heel towards your bottom"	L5, S1 — hamstrings
☐	Knee extension	Knee bent: "Push me away"	L3, L4 — quadriceps
☐	Ankle dorsiflexion	"Pull your foot up, don't let me push it down"	L4, L5 — tibialis anterior
☐	Ankle plantarflexion	"Push down against my hand" (like a pedal)	S1, S2 — gastrocnemius, soleus
☐	Inversion	Hold foot turned in against resistance	L5, S1 — tibialis posterior
☐	Eversion	Hold foot turned out against resistance	L5, S1 — peronei
☐	Great toe extension	"Push your toe up against my finger"	L5, S1 — EHL
☐	Great toe flexion*	"Push your toe down against my finger"	S1, S2 — FHL / FDL
Reflexes — relaxed, positioned; reinforce (clench teeth, Jendrassik) if absent			
☐	Knee jerk	Support both knees slightly flexed; strike patellar tendon	L3, L4
☐	Ankle jerk	Hip externally rotated, knee bent, dorsiflex foot slightly; strike Achilles	S1, S2
☐	Plantar (Babinski)	Blunt object up lateral sole then across; report **first** movement of big toe	Upgoing = UMN
Coordination			
☐	Heel–shin	"Heel on opposite knee, slide down the shin, lift, repeat"	Ataxia, intention tremor
☐	Toe–finger*	"Touch my finger with your big toe"	Past-pointing
☐	Foot tapping	"Tap your foot quickly on my hand"	Dysdiadochokinesia
Sensation — demonstrate on sternum first, eyes closed, compare sides			
☐	Light touch	Cotton wool: dab each dermatome L2–S1	Dermatomal / stocking pattern
☐	Pain	Neurotip: sharp or blunt? Same dermatomes	Spinothalamic loss

	Step	What to do / say	Root / abnormal
☐	Temperature*	Cold tuning fork if indicated	Spinothalamic
☐	Vibration	128 Hz fork on great toe tip; if absent move to MTP → medial malleolus → tibial tuberosity	Dorsal column loss
☐	Proprioception	Hold great toe by its sides; "Up or down?"; if absent move proximally	Dorsal column / sensory ataxia

3. Closing and presenting

☐	Thank patient, help them dress, wash hands
☐	Completion statement: "To complete, I would examine the upper limbs and cranial nerves, examine the spine, and check saddle sensation and anal tone if indicated."
☐	Present one finding per domain: "Gait normal and Romberg's negative; normal bulk; tone normal with no clonus; power 5/5 throughout; reflexes normal, plantars downgoing; coordination normal; sensation intact. Overall a normal lower limb neurological exam."

4. Quick reference

MRC	Power grade
0	No contraction
1	Flicker, no movement
2	Moves with gravity eliminated
3	Moves against gravity, not resistance
4	Against resistance, weaker than normal
5	Normal power

Dermatome	Test site
L2	Anterior upper thigh
L3	Medial knee
L4	Medial leg / medial malleolus
L5	Dorsum of foot, great toe
S1	Lateral foot, little toe, heel
Reflexes	0 absent · 1+ reduced · 2+ normal · 3+ brisk · 4+ clonus

Sign	UMN	LMN
Bulk	Normal / disuse	Wasting
Fasciculations	No	Yes
Tone	Increased, clonus	Decreased
Power	Pyramidal weakness (flexors)	Weak in root/nerve distribution
Reflexes	Brisk, plantar upgoing	Reduced / absent, plantar down

Gait	Think of
Hemiplegic	Circumduction — stroke
Shuffling	Small steps, reduced arm swing — Parkinson's
Broad-based ataxic	Cerebellar lesion
High-stepping	Foot drop — common peroneal / L5
Waddling	Proximal myopathy

- **Pyramidal pattern (UMN):** flexors weaker than extensors in the leg — hip flexion, knee flexion and dorsiflexion go first.
- **Foot drop:** common peroneal = weak dorsiflexion + eversion, **inversion spared**; L5 root = inversion also weak.
- **Romberg's positive** = proprioceptive (dorsal column) loss, not cerebellar. **Stocking** loss = peripheral neuropathy.
- **Red flags:** saddle anaesthesia, urinary retention, bilateral leg weakness = cauda equina — urgent MRI.
- **Pitfalls:** not offering to catch in Romberg's, overpowering the patient, skipping reinforcement, testing sensation with eyes open.

Eye History — OSCE Checklist

Deakin MD2 · BB8 · Ophthalmology

Tick each item as you ask it. **Shaded rows (*)** *are asked if relevant.* † *= added from standard teaching. Use a person-centred approach (Calgary-Cambridge). Ask* **unilateral vs bilateral** *for every symptom.*

1. Opening

☐	Introduce self, confirm patient ID, consent
☐	Open question: "What's brought you in about your eyes today?"

2. Presenting complaint

	Ask about	Example	Significance
☐	Laterality	One eye or both?	
☐	Onset and duration	Sudden or gradual? How long?	Sudden = urgent
☐	Previous episodes	Has this happened before?	Recurrent uveitis, HSV
☐	Vision change	Reduced acuity, blurring, diplopia, field defect	Acute loss = red flag
☐	Flashes and floaters	New flashes, floaters, "curtain" over vision	Retinal tear / detachment
☐	Redness	Where? Diffuse or around the cornea?	Unilateral red eye = red flag
☐	Pain	Severity, constant vs intermittent	Constant pain = red flag
☐	Discharge	Watery, purulent, crusting	Viral vs bacterial conjunctivitis
☐	Photophobia	Light sensitivity	Keratitis, uveitis, meningitis
☐	Sensation	Gritty, foreign-body feeling, itch, dryness	FB, abrasion, allergy, dry eye

3. Eye-specific background

☐	Vision aids	Glasses, contact lenses (overnight wear?)	Contact lens keratitis
☐	Previous eye surgery	Laser, cataract, intravitreal injections, retinal detachment repair	Recent procedure = red flag
☐	Previous trauma	Any past eye injury	
☐	Last review	Last optometry / ophthalmology review	

4. Trauma history*

☐	High-speed injury	Drilling, hammering, grinding, explosion	Beware painful impact then painless eye — penetrating FB
☐	Sharp object	Glass, nail, metal	Globe rupture
☐	Contaminant	Plant material	Fungal keratitis
☐	Chemical	Acid or alkali	Alkali = worst; beware the "white eye"
☐	PPE	Eye protection worn?	

5. Systemic review

	Ask about	Think of
☐	Fever, nausea, vomiting, headache	Acute angle closure, orbital cellulitis
☐	Jaw claudication, scalp tenderness, age >50	Giant cell arteritis
☐	Weight loss, night sweats, rashes, recent viral illness	Systemic or infective cause
☐	Vesicular rash / girdle pain on face	HZO — check Hutchinson's sign (nose tip)
☐	Neurological: speech, vertigo, cognition, balance, weakness, sensory change, diplopia	Stroke, space-occupying lesion

6. Past history, medications, family and social

☐	PMH	Diabetes, CVD, hypertension, autoimmune / connective tissue disease, immunosuppression, HSV/VZV
☐	Medications	COCP, corticosteroids, biologics, α- and β-blockers, anticholinergics, eye drops
☐	Allergies	Drug allergies †
☐	Family history	Glaucoma (incl. angle closure), macular degeneration, strabismus / amblyopia
☐	Social	Smoking, occupation and PPE, **driving**
☐	Risk factors	Age, myopia / hypermetropia, prematurity, previous eye trauma

7. Close

☐	Ideas, concerns, expectations (e.g. impact on driving, work)
☐	Summarise, check accuracy, invite questions, thank patient

Eye Examination — OSCE Checklist

Tick each step as you perform or verbalise it. **Shaded rows (*)** *are optional, verbalised or done if indicated.* † *= added from standard teaching (Talley & O'Connor), not in the Deakin guideline. Order:* **General + vitals → Visual acuity → Visual fields → Pupils → Eye movements → Fundoscopy → Palpation**. *Order may change with the presentation. Always test* **each eye separately**.

1. Introduction

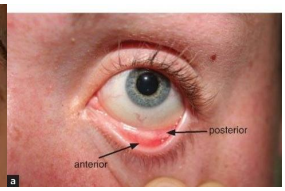
☐	Wash hands; introduce self; confirm patient ID
☐	Explain the exam and gain consent; ask about pain
☐	Position: seated, at eye level, ~1 m apart; glasses available; dim room for pupils/fundoscopy
☐	Equipment: Snellen chart, pinhole, red-headed pin, pen torch, ophthalmoscope ± Ishihara plates

2. Examination

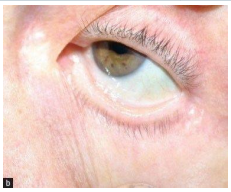
	Step	What to do / say	Abnormal
General inspection and vital signs			
☐	Vital signs	BP, HR, temp etc.	Required in every eye documentation
☐	Ptosis	Compare lid heights	CN III, Horner's, myasthenia
☐	Exophthalmos	Look from front, side, above	Thyroid eye disease, orbital mass
☐	Lids	Swelling, lesions, stye, chalazion, blepharitis	Periorbital / orbital cellulitis
☐	Conjunctiva and sclera	Yellow (jaundice), blue (osteogenesis imperfecta), pallor (anaemia), redness — distribution	Ciliary flush = keratitis, uveitis, glaucoma †
☐	Slit lamp*	Corneal ulceration, anterior chamber (demonstrated in session)	



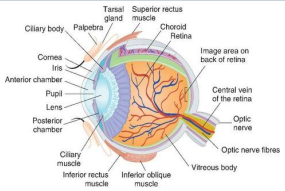
Ptosis



(a) Normal conjunctiva



(b) Conjunctival pallor — anaemia



Anatomy of the eye



Slit lamp

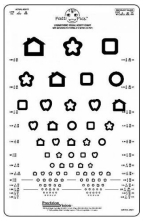
Visual acuity			
☐	Snellen	6 m, glasses on, cover one eye, read smallest line; repeat other eye	Record e.g. 6/9 (distance / line) †
☐	Pinhole	If acuity reduced	Improves = refractive error
☐	Poor acuity*	Count fingers → hand movements → perception of light †	



Snellen chart

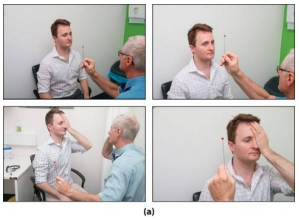


Pinhole occluder

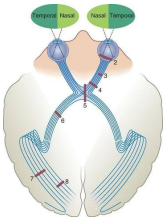


Picture/shape chart (children)

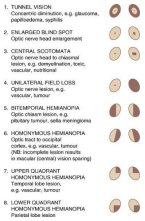
Visual fields (revision from CN exam)			
☐	Confrontation	1 m apart, cover opposite eyes; red pin in from 4 quadrants: "Tell me when you first see red"	Hemianopia, quadrantanopia
☐	Blind spot*	Move pin across centre of field	Enlarged = papilloedema; central scotoma
☐	Red desaturation*	Compare red colour with each eye	Washed-out = optic nerve pathology
☐	Finger wiggle*	If acuity too poor for the pin	Quick screen



Confrontation visual fields



Visual pathway

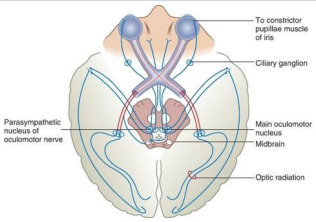


Visual field defects and lesion sites

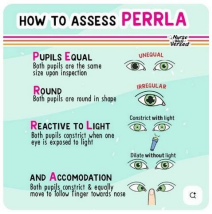
Pupils (dim room, patient looks into distance)			
☐	Inspect	Size (normal 2–6 mm), shape, equality	Anisocoria, irregular pupil
☐	Direct and consensual	Light into each eye in turn; watch both pupils	Afferent vs efferent defect
☐	Swinging light	Swing torch between eyes	RAPD (paradoxical dilation) = optic nerve
☐	Near response	Look far, then at a near target	Constriction + convergence



Direct pupillary reflex

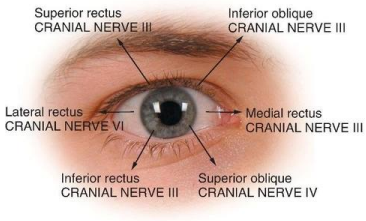


Pupillary light reflex pathway



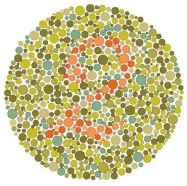
PERRLA

Eye movements (CN III, IV, VI)			
☐	Diplopia	Ask before and during	Binocular diplopia = red flag
☐	H test	Head still; look left, right, up, down; conjugate?	Palsy pattern
☐	Nystagmus	At extremes of gaze	Vestibular, cerebellar
☐	Cover test*	If diplopia: does it go with either eye covered?	Persists with one eye covered = monocular — ask for help



Extraocular muscles and nerves

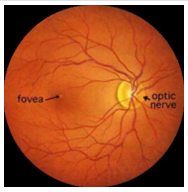
Other tests			
☐	Colour vision*	Ishihara plates (demonstrated in session)	Optic neuritis, congenital deficiency
☐	Corneal reflex*	Rarely routine (ICU / brain death, major trauma)	CN V / VII
☐	Fundoscopy	Red reflex; disc (colour, margins, cup), vessels, macula — dilated pupils in session	Absent red reflex = red flag; papilloedema, retinopathy



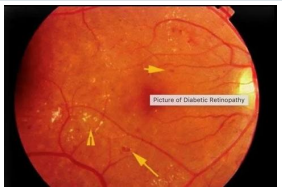
Ishihara colour plate



Ophthalmoscope

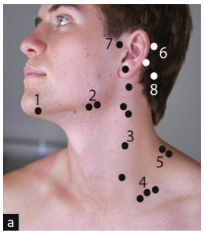


Normal fundus — optic disc and fovea



Diabetic retinopathy

Palpation			
☐	Orbits	Gently palpate orbital rims	Tenderness, step (fracture)
☐	Pre-auricular nodes	Palpate in front of tragus	Viral / chlamydial conjunctivitis



Head and neck lymph node groups

3. Closing and documenting

☐	Thank patient; wash hands
☐	**Everyone**: document vital signs, general inspection, visual acuity, pupils and fundoscopy
☐	**Visual loss / change or neuro signs**: also visual fields, eye movements and cranial nerve examination
☐	Present: "Visual acuity 6/6 both eyes; fields full to confrontation; pupils equal and reactive with no RAPD (PERRLA); full eye movements without diplopia or nystagmus; red reflex present with normal fundi."
☐	Any abnormal eye exam → seek help / refer (optometrist, ophthalmologist, ED)

4. Quick reference

Area	Red flags — refer
Eye	Unilateral red eye, constant pain, acute vision loss, binocular diplopia, abnormal pupil / RAPD, absent red reflex, abnormal cornea, restricted movements
History	Trauma / FB / high-risk activity, recent eye surgery, only seeing eye, previous retinal detachment, neonatal conjunctivitis, non-resolving lid lesion
Associated	Nausea, photophobia, headache, jaw claudication, facial HSV/VZV rash, Hutchinson's sign, painless transient visual loss

Until proven otherwise	
Flashes + floaters	Retinal tear
Vision change / diplopia, age >50	Giant cell arteritis
Serious periocular trauma	Ruptured / penetrated globe; intracranial injury
Periocular laceration	Contains a foreign body
Eyelid laceration	Full thickness, involves globe

• **PERRLA** = Pupils Equal, Round, React to Light, Accommodate.

• Pupil pathway: afferent CN II → pretectal nucleus → both Edinger–Westphal nuclei → efferent CN III (parasympathetic, miosis). Sympathetic (dilation): upper thoracic → superior cervical ganglion → along internal carotid.

ENT Examination — OSCE Checklist

Deakin MD2 · BB4 · Time: 8 min (+2 min reading)

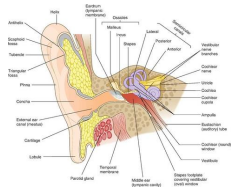
Tick each step as you perform or verbalise it. **Shaded rows (*)** *are optional, verbalised or done if indicated.* † *= added from standard teaching (Talley & O'Connor), not in the Deakin guideline. Order:* **Ears (inspect → palpate → otoscopy → hearing) → Nose (inspect → airflow → rhinoscopy → sinuses) → Throat (mouth → tonsils → neck)**. *Deakin quick reference order. Examine the* **good ear first**.

1. Introduction

☐	Wash hands; introduce self; confirm patient ID
☐	Explain the exam and gain consent; ask about pain
☐	Ask about ear, nose and throat symptoms; position seated, at your eye level
☐	Equipment: otoscope with range of speculae, 512 Hz tuning fork, pen torch, tongue depressor, nasal speculum (or otoscope with wide speculum), gloves

2. Ears

	Step	What to do / say	Abnormal
Ear — inspection (external ear)			
☐	Pinna	Size, shape, deformity; trauma (haematoma)	Cauliflower ear
☐	Pre-auricular region	Cartilage tags, sinuses or pits	Syndromes with hearing loss
☐	Skin	Suspicious lesions (SCC, BCC), skin conditions of pinna and canal	Neoplasia, eczema
☐	Scars	Pre- or post-auricular	Previous surgery
☐	Hearing aids	Note and remove before otoscopy	
☐	Discharge	Mucopurulent, serous, blood-stained	Otitis externa / media, perforation



Anatomy of the ear



Otitis externa



Squamous cell carcinoma of the pinna



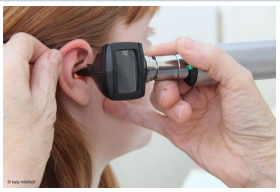
Preauricular sinus

Ear — palpation			
☐	Tragus and pinna	Gently move — tenderness?	Tender = otitis externa
☐	Mastoid	Palpate behind the ear	Tender = mastoiditis (with otitis media)

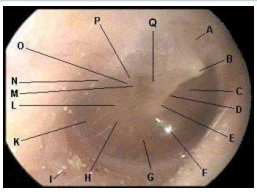


Acute mastoiditis

Ear — otoscopy			
☐	Speculum	Largest that fits comfortably	
☐	Grip	Hold otoscope like a pen; hand braced against patient's head	Prevents injury if head moves
☐	Straighten canal	Adult: pull pinna up and back; child: pull back only	
☐	Canal	Inflamed? Wax? Discharge? Foreign body?	Otitis externa
☐	Tympanic membrane	Colour, light reflex, short process and handle of malleus, umbo, pars flaccida; move to see whole TM	Bulging, retraction, perforation, grommet



Holding the otoscope — hand braced on head



Right tympanic membrane — landmarks



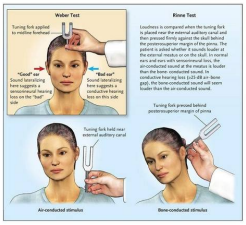
Grommet (ventilation tube)



Common ear drum findings

Ear — hearing			
☐	Whisper test	Mask other ear (rub tragus / fingers); whisper numbers; increase volume if needed; each side	Formal audiology if abnormal
☐	Weber (512 Hz)	Strike fork; base on vertex or midline forehead: "Louder on one side or the middle?"	Lateralises (see table)

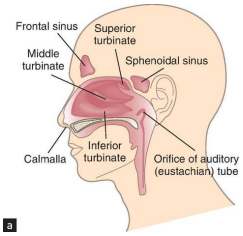
☐	Rinne (512 Hz)	Base on mastoid until no longer heard, then prongs beside the ear: "Can you hear it now?"	Positive (AC > BC) = normal



Weber and Rinne tests

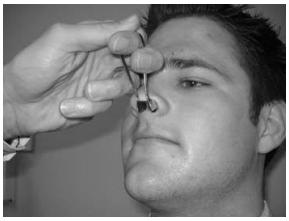
3. Nose and sinuses

	Step	What to do / say	Abnormal
Nose — external inspection			
☐	Front and side	Swelling, deformity, scars or creases, redness / skin disease	Trauma, rhinophyma, BCC
☐	From above	Patient tilts head back; look down the bridge	Deviated nose
☐	Discharge	Rhinorrhoea, crusting; unilateral?	Unilateral offensive in a child = foreign body
☐	Nasal bones	Gently palpate bridge	Tenderness, crepitus, step = fracture



Nasal and sinus anatomy

Nose — airflow			
☐	Misting test	Cold metal tongue depressor under nose as patient breathes out	Condensation under both nostrils = airflow
☐	Sniff test	Occlude one nostril, ask patient to sniff; repeat	Unilateral obstruction
Nose — anterior rhinoscopy (torch ± speculum)			
☐	Without speculum	Tip nose up with thumb; look into the anterior nares with torch	
☐	With speculum / otoscope	Gently open; look straight back, not up	
☐	Septum	Straight or deviated? Bleeding points, clots, crusting, perforation; septal haematoma	Little's area = common epistaxis site †; haematoma = urgent drainage
☐	Turbinates	Inflammation, swelling, colour	Rhinitis (pale, boggy = allergic †)
☐	Polyps*	Grey, glistening, insensate †	Chronic rhinosinusitis



Nasal speculum examination

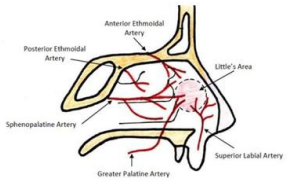


Figure 1. Blood supply to the nasal septum

Blood supply of the septum — Little's area

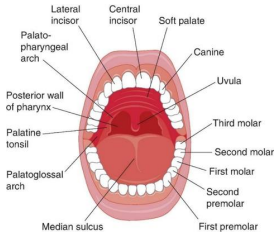


Nasal polyp

Sinuses			
☐	Palpate	Frontal (above brow) and maxillary (cheek) sinuses	Tenderness = sinusitis
☐	Transilluminate*	Pen torch in a dark room (demonstrated in session)	Reduced = fluid / mass
☐	Note	Ethmoid and sphenoid sinuses cannot be examined physically	

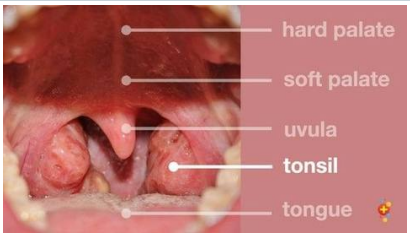
4. Throat and neck

	Step	What to do / say	Abnormal
Mouth — oral cavity (bright pen torch, mouth wide open)			
☐	Lips and mucosa	Ulcers, lesions, colour	Aphthous ulcers, candida, malignancy †
☐	Dentition	Comment on teeth and gums	Dental caries, abscess
☐	Tongue	Appearance; ask to raise tongue to check underneath if indicated	Lesions, floor-of-mouth swelling



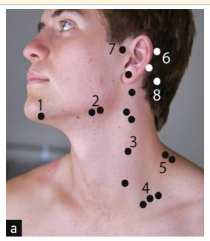
Anatomy of the mouth

Throat — oropharynx			
☐	"Say ahh"	Mouth wide, tongue protruded	Palate rises symmetrically; uvula central †
☐	Tongue depressor	If tonsils not seen: press middle third of tongue, not the back †	
☐	Tonsils	Size, symmetry, inflammation, exudate	Tonsillitis; asymmetry = quinsy or malignancy †
☐	Pharynx	Swelling, inflammation, exudate, post-nasal drip	



Palate, uvula and tonsils

Neck (if clinically indicated)			
☐	Lymph nodes	From behind: submental, submandibular, tonsillar, cervical chain, supraclavicular, pre/post-auricular, occipital	Infection, malignancy
☐	Parotids*	Palpate in front of ear and over angle of jaw	Parotitis, tumour



Cervical lymph node groups

5. Closing and presenting

☐	Thank patient; clean otoscope speculum; dispose of tongue depressor; wash hands
☐	Present: "Pinnae normal and non-tender; canals clear; both TMs intact and pearly grey; whisper test normal, Weber central, Rinne positive bilaterally. No nasal deformity, airways patent, septum midline, no sinus tenderness. Good dentition; tonsils symmetrical without exudate; uvula central; no cervical lymphadenopathy."
☐	Completion: "I would arrange formal audiometry and consider nasendoscopy or a throat swab if indicated."

6. Quick reference

Weber / Rinne (Deakin table)	Weber central	Weber lateralises LEFT	Weber lateralises RIGHT
Rinne AC > BC both ears	Normal	Sensorineural loss RIGHT	Sensorineural loss LEFT
Rinne BC > AC left	—	Conductive loss LEFT	Combined loss
Rinne BC > AC right	—	Combined loss	Conductive loss RIGHT

TM appearance †	Think of
Pearly grey, light reflex	Normal
Red, bulging, lost landmarks	Acute otitis media
Dull, retracted ± fluid level / bubbles	Otitis media with effusion
Hole in TM	Perforation
Small tube in TM	Grommet (not a foreign body)
Swollen canal, debris, tender tragus	Otitis externa

- **Conductive** loss: Weber goes TO the bad ear, Rinne negative (BC > AC).
- **Sensorineural** loss: Weber goes AWAY from the bad ear, Rinne positive (both reduced).
- **False-negative Rinne**: a dead ear hears BC louder via the other cochlea — Weber clarifies.
- **Quinsy †**: unilateral tonsil swelling, uvula deviated away, trismus, "hot potato" voice — urgent ENT.
- **Airway red flags †**: stridor, drooling, can't swallow saliva — don't use a tongue depressor; call for help.

Cardiovascular Examination — OSCE Checklist

Deakin MD2 · CR3 · Time: 8 min (+2 min reading)

Tick each step as you perform or verbalise it. **Shaded rows (*)** *are optional, verbalised or done if indicated.* † *= added from standard teaching (Talley & O'Connor), not in the Deakin guideline. Order:* **General → Vitals → Hands → Arms (pulse, BP) → Face → Neck (carotid, JVP) → Praecordium → Back → Abdomen → Legs → Other**. *Patient at* **45°**. *Expose the chest only when needed.*

1. Introduction

☐	Hand hygiene; introduce self; confirm patient ID
☐	Explain the exam and gain consent: upper body clothing off, exposed only when needed
☐	Offer a chaperone; ensure privacy, comfort and warmth
☐	Position: on the bed at **45°**, supported with pillows
☐	Equipment: stethoscope, sphygmomanometer, torch, ruler †

2. General inspection and vital signs

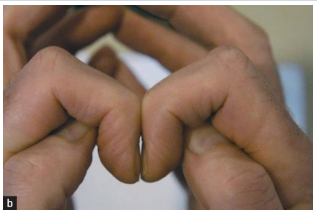
	Step	What to do / say	Abnormal
☐	Well or unwell?	Pallor, jaundice, cachexia, sweaty/diaphoretic	Shock, heart failure, endocarditis
☐	Breathing	Respiratory effort, rate, distress	Pulmonary oedema
☐	Body features	Marfan's, Turner's, trisomy 21	Associated cardiac lesions
☐	Mobility, expression	Walked in? How? Anxious or in pain?	
☐	Bedside clues	Medications, glucometer/CGM, oxygen, IV cannula/fluids, 'fluid restriction' or 'low salt' sign, cigarettes/vapes	
☐	Vital signs	HR, BP, RR, temp, SpO_2, BGL	

3. Hands and arms

	Step	What to do / say	Abnormal
☐	Clubbing	Schamroth's window (nail-fold angle)	Infective endocarditis, cyanotic heart disease
☐	Splinter haemorrhages	Nail beds	Infective endocarditis
☐	Osler's nodes	Red, raised, **painful** 0.5–1 cm nodules on finger/toe pulps, thenar/hypothenar	Infective endocarditis
☐	Janeway lesions	**Non-tender** red lesions on palms or finger pulps	Infective endocarditis
☐	Colour	Peripheral cyanosis; nicotine staining; pallor of palmar creases	Low output; smoking; anaemia
☐	Temperature	Warm (vasodilated), sweaty, tremor; or cool, peripherally shut down	Sepsis, thyrotoxicosis; shock
☐	Tendon xanthoma	Yellow deposits over extensor tendons	Hypercholesterolaemia
☐	Radial pulse	Rate and rhythm	Tachy-/bradycardia; irregularly irregular = AF
☐	Radio-radial delay	Both radials together: timing and volume	Aortic dissection, subclavian stenosis
☐	Radio-femoral delay	Radial + femoral together	Coarctation of the aorta
☐	Arms	IV drug injection scars	Endocarditis risk
☐	Blood pressure	Manual BP, **both arms**, **lying and standing**	Hypertension; postural drop



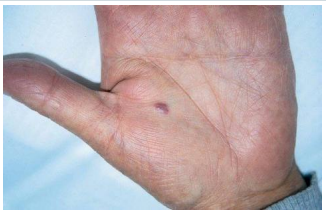
Schamroth's sign — clubbed (no window)



Normal — diamond window



Splinter haemorrhages

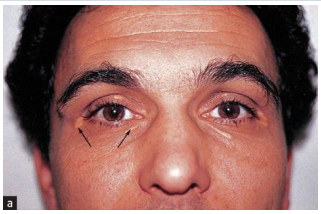


Janeway lesion

4. Face and neck

	Step	What to do / say	Abnormal
☐	Face	Expression (pain, apprehension); mitral facies (malar flush)	Mitral stenosis
☐	Eyes	Xanthelasmata, corneal arcus; jaundice; conjunctival pallor †	Hyperlipidaemia; congestive hepatopathy; anaemia
☐	Mouth	Central cyanosis; high-arched palate; poor dentition	Hypoxaemia; Marfan's; endocarditis source
☐	Carotid pulse	**One side at a time**: fingers behind neck, thumb at/below cricoid; volume and character	Slow-rising (AS), collapsing (AR)
☐	Carotid bruit	Bell; patient holds breath; each side; tell them to breathe again	Carotid stenosis
☐	JVP — position	45°, head tilted up and turned slightly away; look at **internal** jugular (between heads of SCM)	
☐	JVP — see it	Torch at an oblique angle; look for the **double flicker**	

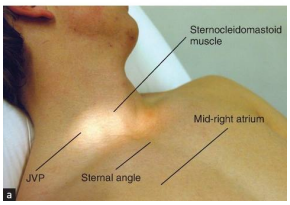
	Step	What to do / say	Abnormal
☐	JVP — measure	Ruler held horizontally from top of pulsation; measure vertical height above sternal angle	>3 cm = raised (right heart failure, fluid overload)
☐	JVP — character	Waveform	Large v waves (TR), absent a waves (AF) †



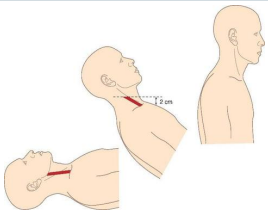
Xanthelasmata



Corneal arcus



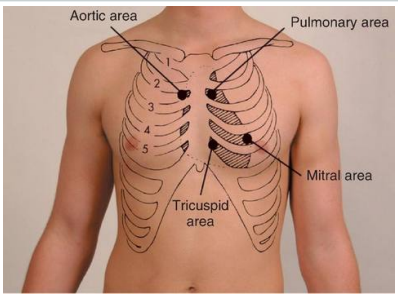
JVP and sternal angle



JVP height changes with position

5. Praecordium

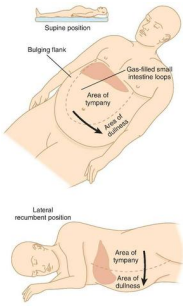
	Step	What to do / say	Abnormal
Inspect			
☐	Scars	Median sternotomy, thoracotomy †	Previous cardiac surgery
☐	Chest wall	Pectus excavatum, other deformity	
☐	Devices	Pacemaker or defibrillator box	
☐	Pulsations	Visible apex beat	
Palpate			
☐	Apex beat	Supine or left lateral: normally **5th ICS, mid-clavicular line**; presence, position, character	Displaced = cardiomegaly; heaving = pressure-loaded; diffuse = volume-loaded
☐	Absent apex?	Consider thick chest wall, emphysema, pericardial effusion, dextrocardia	
☐	Parasternal impulse	Heel of hand at left sternal edge	Heave = right ventricular enlargement
☐	Thrills	Flat hand over apex, left sternal edge, then base (aortic and pulmonary areas)	Palpable murmur; time with apex beat
Auscultate (time with carotid pulse †)			
☐	Mitral area	Apex: 5th left ICS, mid-clavicular line	S1, S2; murmurs
☐	Tricuspid area	5th left ICS, at the left sternal edge	
☐	Pulmonary area	2nd left ICS	
☐	Aortic area	2nd right ICS	
☐	Heart sounds	S1 (mitral + tricuspid closure), S2 (aortic + pulmonary); added sounds?	Murmur: systolic or diastolic, grade (Levine)
☐	Manoeuvres*	Left lateral + bell at apex; sit forward in full expiration at left sternal edge †	Mitral stenosis; aortic regurgitation



Auscultation areas

6. Back and abdomen

	Step	What to do / say	Abnormal
☐	Back	Sit forward: inspect for scars and deformity	Previous surgery
☐	Lung bases	Percuss and auscultate	Crackles, effusion (heart failure)
☐	Sacral oedema	Press over sacrum	Fluid overload
☐	Liver	Start in RIF, hand parallel to costal margin, advance on each breath out; span (normal <13 cm)	Enlarged, tender (congestion); pulsatile (TR †)
☐	Spleen	Lower mid-abdomen towards left costal margin; roll patient towards you	Splenomegaly (endocarditis)
☐	Ascites	Percuss midline → flanks; if dull, test shifting dullness (roll, wait 30–60 s)	Right heart failure
☐	Aorta	Palpate left of midline	Expansile = AAA
☐	Femoral arteries	Palpate and auscultate (midway pubic symphysis–ASIS)	Weak pulse, bruit (PVD)



Shifting dullness



Peripheral pulses

7. Legs

	Step	What to do / say	Abnormal
☐	Inspect	Scars (vein harvest †), ulcers, hair loss, shiny skin, trophic nails, colour (white, cyanosed, red), clubbing of toes	Peripheral vascular disease
☐	Oedema	Ankle swelling; press for pitting; note the upper level	Heart failure
☐	Calves	Tenderness, swelling †	DVT
☐	Popliteal pulse	Centre of popliteal fossa, knee slightly bent	
☐	Posterior tibial	Behind medial malleolus	Absent = PVD
☐	Dorsalis pedis	Lateral to extensor hallucis longus tendon	Absent = PVD
☐	Capillary return	Press toe, watch colour return	Slow = poor perfusion
☐	Buerger's test*	If slow: raise legs until pale, then hang over the side of the bed	Delayed colour, cyanosis = arterial insufficiency

8. Closing and presenting

☐	Completion: "I would also do urinalysis and fundoscopy, and review the ECG and observation chart †."
☐	Simple explanation of findings; anything you can't answer → "I'll pass that on to someone senior"
☐	Ask about concerns/questions; check understanding and comfort; patient can redress
☐	Hand hygiene; return equipment; document findings
☐	Present: "Mr X is comfortable at rest; pulse 72 regular, BP 125/80. No peripheral stigmata of endocarditis or hyperlipidaemia. JVP not raised. Apex beat 5th ICS mid-clavicular line, undisplaced. Heart sounds dual with no added sounds. Lung bases clear, no peripheral oedema, peripheral pulses present."

9. Quick reference

Category	Systolic	Diastolic
Optimal	<120 and	<80
Normal	120–129 and/or	80–84
High normal	130–139 and/or	85–89
Grade 1 HTN	140–159 and/or	90–99
Grade 2 HTN	160–179 and/or	100–109
Grade 3 HTN	≥180 and/or	≥110
Isolated systolic	≥140 and	<90

Grade	Levine murmur grading
1/6	Very soft, not heard at first
2/6	Soft, heard almost immediately
3/6	Moderate, no thrill
4/6	Loud, thrill just palpable
5/6	Very loud, thrill easily palpable
6/6	Heard without stethoscope on chest

Murmur †	Timing	Best heard	Key signs
Aortic stenosis	Ejection systolic	Aortic area → carotids	Slow-rising pulse, narrow pulse pressure
Mitral regurgitation	Pansystolic	Apex → axilla	Displaced apex, AF
Aortic regurgitation	Early diastolic	Left sternal edge, sitting forward, expiration	Collapsing pulse, wide pulse pressure
Mitral stenosis	Mid-diastolic rumble	Apex, left lateral, bell	Malar flush, AF, tapping apex

- **Heart failure †**: tachycardia, raised JVP, displaced apex, S3, bibasal crackles, pleural effusion, tender liver, sacral and ankle oedema.
- **Infective endocarditis**: clubbing, splinters, Osler's nodes, Janeway lesions, new murmur, splenomegaly, fever.
- BP: one reading cannot diagnose hypertension (classification for reference only).

Respiratory Examination — OSCE Checklist

Deakin MD2 · CR7 · Time: 8 min (+2 min reading)

Tick each step as you perform or verbalise it. **Shaded rows (*)** *are optional, verbalised or done if indicated.* † *= added from standard teaching (Talley & O'Connor), not in the Deakin guideline. Order:* **Environment → General → Vitals → Hands → Arms → Face → Trachea → Nodes → Posterior chest → Anterior chest**. *Deakin order: examine the* **back first**, *then the front. Always compare side to side.*

1. Introduction

☐	Wash hands; introduce self; confirm patient ID
☐	Explain and gain consent: upper body clothing removed, only exposed when needed
☐	Offer a chaperone; privacy; warmth
☐	Position: sitting at 45° † (sit forward for the back)
☐	Equipment: stethoscope

2. Examination

	Step	What to do / say	Abnormal
Environment and general inspection			
☐	Environment	Oxygen (device, flow), inhalers, spacers, sputum pot, nebuliser †	Chronic lung disease
☐	Respiratory effort	Rate, accessory muscle use, tripod position, pursed lips †	Respiratory distress
☐	Speech	How much can they say in one breath?	Full sentences vs words
☐	Audible sounds	Cough, wheeze, stridor, voice change	Obstruction, recurrent laryngeal nerve
Vital signs			
☐	Full set	RR (count 30 s+), SpO_2 (on air?), HR, BP, temp	
Hands			
☐	Nails	Clubbing (look side-on, Schamroth's window †)	Lung cancer, bronchiectasis, ILD, CF
☐	Nicotine staining	Fingers and nails	Smoking
☐	Palms	Pallor	Anaemia
☐	Tremor	Fine tremor; flap if CO_2 retention †	Salbutamol use; hypercapnia
Arms			
☐	Skin	Bruising, thin skin	Long-term corticosteroids
Face			
☐	Eyes	Conjunctival pallor; Horner's syndrome (ptosis, miosis)	Anaemia; Pancoast tumour
☐	Sinuses	Facial tenderness	Sinusitis
☐	Mouth	Central cyanosis (under tongue)	Hypoxaemia
Neck			
☐	Trachea	Midline position (warn patient); tracheal tug	Deviation: pneumothorax, collapse, effusion
☐	Lymph nodes (from behind)	8 groups: submental, submandibular, anterior cervical, posterior cervical, pre-auricular, post-auricular, occipital, supraclavicular	Malignancy, infection
☐	JVP*	Raised JVP †	Cor pulmonale
Posterior chest (patient sitting forward, arms crossed †)			
☐	Inspect	Shape, symmetry, scars (thoracotomy, drains), muscle wasting, kyphoscoliosis	Previous surgery, restrictive disease
☐	Expansion	Hands on lower ribs, thumbs meeting in midline; deep breath — watch thumbs separate †	Reduced side = pathology
☐	Rib tenderness	Palpate ribs laterally	Fracture, costochondritis
☐	Percussion	Compare side to side, top to bottom, including axillae †	Dull: consolidation, collapse; stony dull: effusion; hyper-resonant: pneumothorax
☐	Breath sounds	Diaphragm, side to side, top to bottom; patient breathes through mouth	Vesicular vs bronchial; reduced; added sounds
☐	Added sounds	Crackles (fine/coarse), wheeze, pleural rub	Fibrosis, oedema, infection; airway narrowing
☐	Vocal resonance	"Say 99" while auscultating each area †	Increased = consolidation; reduced = effusion
Anterior chest (patient back at 45°)			
☐	Inspect	Look again for increased work of breathing, chest shape (barrel, pectus), scars	Hyperinflation (COPD)
☐	Palpate	Subcutaneous emphysema (crackling under fingers); apex beat †	Air leak; mediastinal shift
☐	Percussion	Include supraclavicular region and clavicles	As above
☐	Auscultation	Breath sounds, added sounds, vocal resonance including apices	As above
Legs*			
☐	Oedema, calves	Pitting oedema; calf swelling/tenderness †	Cor pulmonale; DVT (PE risk)

3. Closing and presenting

☐	Help patient dress; leave them as they arrived; wash hands
☐	If asked a question you can't answer, say you'll pass it to someone senior
☐	Completion: "I would review the observation chart, do a peak flow / spirometry, examine sputum, and request a chest X-ray †."
☐	Present: "Mrs X is comfortable at rest on room air, RR 14, SpO_2 98%. No clubbing or peripheral signs of respiratory disease. Trachea midline, expansion equal, percussion resonant throughout, vesicular breath sounds with no added sounds."

4. Quick reference †

Condition	Expansion	Trachea	Percussion	Breath sounds	Vocal resonance
Consolidation (pneumonia)	↓ affected side	Midline	Dull	Bronchial ± crackles	↑
Pleural effusion	↓ affected side	Away (if large)	Stony dull	↓ or absent	↓
Pneumothorax	↓ affected side	Away (tension)	Hyper-resonant	↓ or absent	↓
Collapse (lobar)	↓ affected side	Towards	Dull	↓	↓
COPD	↓ both, hyperinflated	Midline (tug)	Hyper-resonant	↓ ± wheeze	Normal / ↓
Pulmonary fibrosis	↓ both	Midline	Normal	Fine end-inspiratory crackles	Normal / ↑

- **Stridor** = upper airway (inspiratory); **wheeze** = lower airway (expiratory). Stridor is an emergency.
- Clubbing is **not** a feature of uncomplicated COPD — look for another cause (e.g. lung cancer).

Abdominal (GI) Examination — OSCE Checklist

Deakin MD2 · RG5 · Time: 8 min (+2 min reading)

Tick each step as you perform or verbalise it. **Shaded rows (*)** *are optional, verbalised or done if indicated.* † *= added from standard teaching (Talley & O'Connor), not in the Deakin guideline. Order:* **General → Vitals → Hands → Arms → Face → Chest → Abdomen (inspect, palpate, percuss, auscultate) → Hernias → Complete**. *Watch the patient's face throughout and examine any painful area* **last**.

1. Introduction

☐	Wash hands; introduce self and the exam; confirm patient ID
☐	Explain and gain consent; ask about pain (examine that area last)
☐	Offer a chaperone; privacy (curtains); quiet, well-lit, warm room
☐	Position: lying flat on **one pillow**, arms by sides
☐	Expose abdomen; cover upper and lower body with 2 blankets (ideally nipples to mid-thigh †)
☐	Equipment: stethoscope, tape measure, torch

2. Examination

	Step	What to do / say	Abnormal
General inspection			
☐	Jaundice	Skin and sclera	Liver disease, haemolysis, obstruction
☐	Skin	Discoloration, pigmentation, rashes, acanthosis nigricans	
☐	Pain and position	Lying very still / writhing / sitting forward	Peritonitis / renal colic / pancreatitis
☐	Body habitus	Wasting or obesity	Malignancy, malabsorption
☐	Mental state	Confusion	Hepatic encephalopathy



Scleral icterus (jaundice)

Vital signs			
☐	Pulse (manual)	Rate, rhythm, character	Tachycardia: sepsis, bleeding
☐	BP lying and standing	Postural drop	Hypovolaemia (GI bleed)
☐	RR, temp, SpO_2, BGL	Full set; height and weight if possible	
Hands (both)			
☐	Nails	Leukonychia, clubbing	Hypoalbuminaemia; IBD, cirrhosis
☐	Palms	Pale palmar creases; palmar erythema	Anaemia; liver disease (may be normal)
☐	Dupuytren's contracture	Thickened palmar fascia, fixed finger flexion	Alcohol-related liver disease
☐	Asterixis	Arms out, wrists cocked back, fingers spread, ≥20 seconds	Liver failure ("flap")



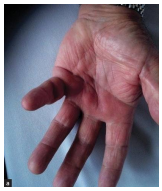
Leukonychia



Clubbing



Palmar erythema



Dupuytren's contracture

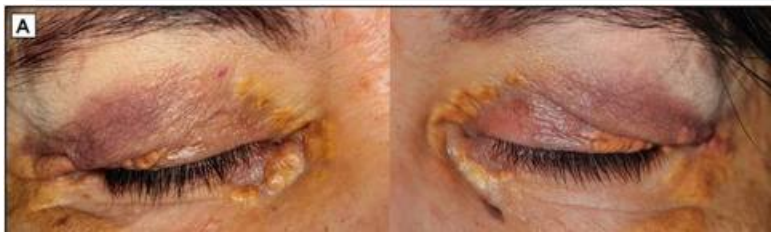
Arms			
☐	Skin	Bruising, petechiae, scratch marks, spider naevi	Coagulopathy, cholestasis (pruritus), chronic liver disease
☐	Muscle	Wasting	Malnutrition



Spider naevi

Face	

☐	Eyes	Pale conjunctiva, scleral jaundice, xanthelasma	Anaemia; liver disease; cholestasis
☐	Salivary glands	Parotid and submandibular enlargement	Alcohol, sialadenitis
☐	Mouth (torch)	Angular stomatitis, dentition, oral candidiasis	Iron/B deficiency, immunosuppression
☐	Lymph nodes*	Left supraclavicular (Virchow's) node †	Gastric malignancy (Troisier's sign)

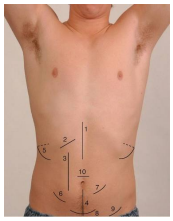


Xanthelasma



Angular stomatitis

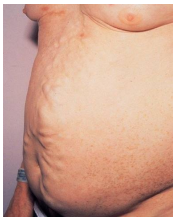
Chest			
☐	Spider naevi	>5 in SVC distribution †	Chronic liver disease
☐	Gynaecomastia*	Verbalise — not expected to examine	Chronic liver disease, spironolactone
Abdomen — inspection (stand back, then from the side)			
☐	Scars, stoma	Surgical or trauma scars; laparoscopic ports	Previous surgery
☐	Distension	5 Fs: fat, fluid, faeces, flatus, fetus	Ascites, obstruction
☐	Localised swelling	Mass, bowel loop, enlarged organ	
☐	Veins and pulsation	Prominent veins; visible pulsation or peristalsis	Portal hypertension; AAA; obstruction
☐	Cough*	Ask patient to cough — watch hernial orifices and for pain †	Hernia; peritonism



Abdominal scars (see quick reference)

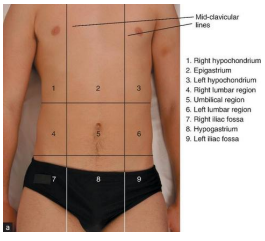


Gross ascites



Distension and prominent veins

Abdomen — palpation (warm hands, kneel to bed level †, watch face)			
☐	Light palpation	All 9 regions, flat of hand, painful area last	Tenderness, guarding, rigidity, rebound
☐	Deep palpation	All 9 regions again	Masses — describe (see box)
☐	Liver	Start RIF, hand parallel to right costal margin; patient breathes in through mouth; move up 1–2 cm on each breath out	Edge: hard/soft, tender, regular, pulsatile
☐	Spleen	Left hand behind left lower ribs; right hand from RIF towards left costal margin	Any palpable spleen is abnormal
☐	Kidneys	Bimanual ballot each side	Normal kidneys are impalpable
☐	Aorta	Both hands either side above umbilicus	Expansile + pulsatile = AAA
☐	Bladder*	Palpate suprapubic area if retention suspected	Dull to percussion



The nine abdominal regions

Abdomen — percussion			
☐	Liver span	Right MCL from above 6th rib (resonant → dull) down to palpable edge	Normal <12 cm
☐	Spleen*	Percuss left costal margin / Traube's space †	
☐	Shifting dullness	Percuss from midline to flank until dull; mark; roll patient towards you, wait 30 s, re-percuss †	Dullness shifts = ascites
☐	Bladder*	If urinary retention suspected	Dullness above pubis
Abdomen — auscultation			
☐	Bowel sounds	Just below umbilicus	Absent (ileus, peritonitis) / tinkling (obstruction †)
☐	Renal bruits	Either side of midline at level of umbilicus (check BP if present)	Renal artery stenosis
☐	Aortic bruit*	Above umbilicus †	Atherosclerosis, AAA

Hernias and groin			
☐	Hernial orifices	Strain or cough: midpoint of inguinal ligament and lateral to pubic tubercle; examine standing	Inguinal / femoral hernia
☐	Inguinal lymph nodes	Both sides	Lymphadenopathy
Legs*			
☐	Oedema	Ankle and sacral pitting oedema †	Hypoalbuminaemia, liver disease

3. Closing and presenting

☐	Completion: "I would also examine the external genitalia, perform a DRE, and dipstick the urine."
☐	Ask about concerns/questions; check understanding
☐	Cover patient, help them dress, leave them comfortable
☐	Put equipment away; wash hands
☐	Present: "Mr X is comfortable at rest with no peripheral stigmata of chronic liver disease. The abdomen is soft and non-tender with no organomegaly or masses, no shifting dullness, and normal bowel sounds."

4. Quick reference

Mass — describe	
Site	Region
Size, shape	cm
Surface, edge	Regular / irregular
Consistency	Hard / soft
Tenderness	
Mobility	Mobile / fixed; moves with breathing?
Pulsatile?	Expansile = aneurysm
Get above it?	Can't get above liver or spleen
Percussion	Resonant / dull

Scar	Operation
Upper midline	Laparotomy (upper GI)
Right subcostal (Kocher's)	Open cholecystectomy
Right paramedian	Older laparotomy
Lower midline	Laparotomy (lower GI, gynae)
Loin	Nephrectomy
RIF gridiron	Appendicectomy
Iliac fossa J-shape	Kidney transplant
Pfannenstiel	Caesarean, gynae
Umbilical port	Laparoscopic surgery

Spleen vs left kidney †	Spleen	Kidney
Notch	Present	Absent
Get above	No	Yes
Moves with breath	Down and medially	Little
Ballotable	No	Yes
Percussion	Dull	Resonant (bowel in front)

- **Hepatomegaly causes †**: congestion (heart failure), infiltration (fatty liver, malignancy), infection (hepatitis).
- **Splenomegaly causes †**: portal hypertension, haematological malignancy, infection (EBV, malaria).
- **Chronic liver disease signs**: clubbing, leukonychia, palmar erythema, Dupuytren's, spider naevi, gynaecomastia, jaundice, ascites, caput medusae.
- Pitfalls: more than one pillow, not watching the face, palpating tender area first, forgetting hernias.

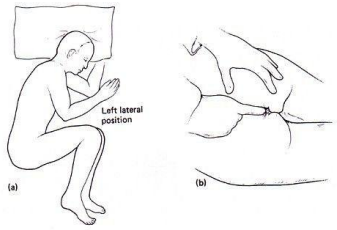
Digital Rectal Examination (DRE) — OSCE Checklist

Deakin MD2 · RG5 · Part-task trainer

Tick each step as you perform or verbalise it. **Shaded rows (*)** *are optional, verbalised or done if indicated.* **†** *= added from standard teaching (Talley & O'Connor), not in the Deakin guideline. Order:* **Consent → Position → Inspect → Strain → Insert → Sweep walls → Anterior wall → Tone → Inspect glove → Close**. *Treat the model as a real patient: communicate, reassure and explain what they will feel.*

1. Before you start

☐	Wash hands; fingernails short; introduce self; confirm patient ID
☐	Informed consent **before** patient undresses: why, what you will do, position, what they will feel
☐	Do not assume willingness because they have not objected; allow time for questions
☐	Offer a chaperone; privacy (curtains); quiet, well-lit, warm room
☐	Check cautions — seek senior advice if: anal fissure, thrombosed prolapsed haemorrhoids, child, active IBD / diverticulitis, recent surgery/trauma/radiotherapy, immunocompromised or bleeding risk, history of abuse
☐	Equipment: **2 pairs** of non-sterile gloves, lubricant, light, bluey, tissues
☐	Position: **left lateral**, knees drawn up to chest; cover with gown/sheet until exposure needed



Left lateral position for DRE

2. Procedure

	Step	What to do / say	Abnormal
Inspect (gloved, separate buttocks)			
☐	Perianal area	Skin tags, erythema, excoriation, warts (condylomata), fissure, external/thrombosed haemorrhoids, prolapse, carcinoma	
☐	Strain	"Push down as if opening your bowels"	Incontinence, perineal descent, haemorrhoids, prolapse
Insert			
☐	Warn the patient	"You may feel like you need to open your bowels — it's just the feeling of my finger"	
☐	Lubricate	Lubricate right index finger; lift right buttock with left hand	
☐	Position finger	Pad of finger on anus, pointing towards patient's front	
☐	Relax and enter	Patient breathes deeply through mouth; wait for sphincter to relax, then gently insert, turning finger towards head	Internal sphincter tone assessed on entry
☐	Resistance	Stop, hold still, let patient breathe slowly; do **not** force. Stop if spasm and severe pain	Fissure, stricture, fibrosis, carcinoma
Palpate			
☐	Rectal walls	Sweep pulp from 2 o'clock clockwise to 10 o'clock at 3 levels (~7–8 cm); feel sacral hollow and coccyx	Mass, polyp, ulcer with hard raised edge
☐	Anterior wall — male	10 to 2 o'clock: prostate size, consistency, median sulcus, nodules, tenderness ("you may feel you need to pass urine")	See box
☐	Anterior wall — female	Cervix felt as a smooth lump; retroverted uterus may press on wall	Pelvic mass
☐	Strain again	Patient pushes down; sweep to feel an extra centimetre	
☐	Contents	Consistency of faeces	Hard impacted stool, foreign body
☐	Squeeze	"Relax… now squeeze my finger… now relax"	External sphincter tone; weak = sphincter injury or neurological
After removal			
☐	Inspect glove	Colour, blood, mucus, pus	Fresh blood = haemorrhoids, rectal lesion; black = upper GI bleed, iron
☐	FOBT*	Faecal occult blood test if indicated	

3. Closing

☐	Offer tissues / help wiping; remove outer gloves when soiled
☐	Dispose of waste in contaminated bin
☐	Cover patient; tell them it's finished; offer help dressing
☐	Simple explanation of findings; if unsure, say you'll confirm with the treating doctor
☐	Invite questions; check understanding; wash hands
☐	Document consent, chaperone and all findings

4. Quick reference

Finding	Think of
Loss of tone, patulous anus	Sphincter injury, cord compression / cauda equina
Hard impacted stool	Constipation, overflow incontinence
Irregular mucosa / mass	Rectal cancer
Smooth, large, firm, non-tender prostate	BPH
Hard irregular nodule / fixed mass	Prostate cancer, chronic prostatitis
Large, boggy, tender prostate	Acute prostatitis
Mass on anterior wall (female)	Ovarian or uterine mass

#	Indications
1	Rectal bleeding
2	Anal symptoms: itch, pain, burning
3	Change in bowel habit
4	Urinary symptoms (prostate)
5	Incontinence
6	Suspected spinal cord compression — check anal tone

• Normal prostate: rubbery, firm, bilobed, smooth with a central median sulcus. With age it enlarges and the sulcus is lost; cancer is hard and irregular.
• Internal sphincter = smooth muscle (involuntary, on entry). External sphincter = skeletal muscle (voluntary, "squeeze").